

Symptoms and Treatment of Overdose:

Sign of overdosage may appear shortly after ingestion and they include: Diarrhoea, vomiting or nausea, loss of appetite, stomach cramps or pain, unusual increase sweating. If overdosage is suspected seek medical assistance or contact a poison control centre immediately.

Interactions with Other Medicaments:

Cholestyramine: The absorption of paracetamol is reduced by cholestyramine. Therefore, the cholestyramine should not be taken within one hour if maximal analgesia is required. **Metoclopramide** and **Domperidone:** The absorption of paracetamol is increased by metoclopramide and domperidone. However, concurrent use need not be avoided. **Warfarin:** Potentiation of warfarin with continual high dosage of paracetamol. **Chloramphenicol:** Increased plasma concentration of chloramphenicol. **Busulfan:** Paracetamol possibly inhibits metabolism of intravenous busulfan.

Storage Condition:

Keep container tightly closed. Store in a dry place (below 30°C). Protect from light.

Pack Size:

Blister pack of 1x10s, 3x10's, 20x10's, 100x10's and 630x10's.

Product Registration Number:

MAL 07091034XZ

Further information can be obtained from your doctor or pharmacist.

ROYCE®

Product Holder / Manufactured by:

ROYCE PHARMA MFG SDN BHD. 650435-X
PT 1663, Nilai Industrial Estate,
71800 Nilai, Negeri Sembilan, Malaysia.

Revision date: 150917

ROYCE®

Redamol Tablet

(Paracetamol – Analgesic & Antipyretic)

Presentation:

White round tablet, bevel-edge with breaking line on one side and plain on the other side.

Content:

Each tablet contains: Paracetamol 500 mg.

Indication:

For relief of mild to moderate pain and to reduce fever. It provides symptomatic relief only; additional therapy to treat the cause of the pain or fever should be instituted when necessary.

Pharmacological Information:

Analgesic - The mechanism of analgesic action has not been fully determined. Paracetamol may act by inhibiting prostaglandin synthesis in the central nervous system (CNS) and through a peripheral action by blocking pain-impulse generation. The peripheral action may also be due to inhibition of the synthesis of prostaglandin or to inhibit the synthesis or action of other substances, which sensitize pain receptors to mechanical or chemical stimulation.

Antipyretic - Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat-regulating center to produce peripheral vasodilation resulting in increased blood flow through the skin, sweating and heat loss. The central action probably involves inhibition of prostaglandin synthesis in the hypothalamus.

Paracetamol have analgesic and antipyretic effects similar to those of aspirin. However, they have only weak anti-inflammatory effects and do not share the antirheumatic uses of the Salicylates. The pharmacological effects of administered phenacetin are a combination of its inherent effects (Conney et al., 1966) and those of paracetamol, its major metabolite. Minor metabolites contribute significantly to the toxic effects of both drugs.

The pharmacological properties of paracetamol and phenacetin have been reviewed by Smith (1958), Randall (1963) and Beaver (1965, 1966).

Analgesic and Antipyretic, paracetamol relieve pain of moderate intensity, such as usually occurs in headache and dysmenorrhea, and many muscle joint, and peripheral nerve disorders. Intense pain or that arising from smooth muscle spasm in hollow viscera is not alleviated. Paracetamol reduce fever by a direct effect on the heat-regulating centers to increase dissipation of body heat. Paracetamol reduces fever not by interfering with the release of endogenous pyrogen from leukocytes but by inhibiting the action of endogenous pyrogen on the pyrothalamal heat-regulating centers. The locus of the analgesic effect is uncertain. Laboratory tests have been interpreted to indicate that it is solely a central effect, solely a peripheral effect, or a combination of both. The mechanism of the antipyretic and analgesic effects is also uncertain. Paracetamol is more active than aspirin as an inhibitor of prostaglandin synthetase of brain, but it is only a very weak inhibitor of prostaglandin synthesis by a preparation from spleen.

Absorption, distribution, biotransformation and excretion.

Paracetamol and phenacetin are metabolized primarily by the hepatic microsomal enzymes. Paracetamol is rapidly and practically completely absorbed from the gastrointestinal tract. Plasma concentration reaches a peak in $\frac{1}{2}$ to 1 hour; the plasma half-time is 1 to 3 hours. Paracetamol is relatively uniformly distributed throughout most body fluids. Binding of the drug to plasma proteins is variable; 20% to 50% may be bound at the concentrations encountered during acute intoxication. About 3% of paracetamol is excreted unchanged in the urine, and about 80% is excreted in the urine after conjugated in the liver, predominantly with glucuronic acid and to small extent with sulfuric acid. The glucuronide accumulates in plasma before excretion. A conjugate with cysteine and metabolites produced by hydroxylation and deacetylation have also been detected. The hydroxylated metabolites are responsible for methemoglobin formation and hepatotoxicity.

Administration of paracetamol to patients with impaired renal function results in increased accumulation of conjugated paracetamol in plasma but only minor changes in the plasma concentrations of phenacetin and free paracetamol.

Dosage and Administration:

Adults: One to two tablets.

Children 7 to 12 years: Half to one tablet. To be taken three or four times a day. Not recommended for children under 6 years.

Contraindication:

Use of this medicine should be carefully considered when the following problem exists: Alcoholism, Hepatic disease, Viral infection, Renal function impairment and Cardiovascular disease.

Warning and Precautions:

For pregnant women and nursing mothers, consult your physician before taking this or any medicine.

Caution:

May cause renal impairment. Should be given with care to patients with impaired kidney or liver function.

This preparation contains PARACETAMOL. Do not take any other PARACETAMOL containing medicine at the same time.

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Pregnancy and Lactation:

There is clinical and epidemiological evidence of safety of paracetamol in pregnancy. Paracetamol is not known to be harmful to pregnant women. Paracetamol is excreted in breast milk but not in a clinically significant amount. Available published data do not contraindicate breast feeding.

Side Effects:

Incidence of side effects are rare, but if any of the following symptoms occur, consult your physician immediately:

- Bloody or cloudy urine - Difficult or painful urination
- Frequent urge to urinate - Sudden decrease amount of urine
- Skin rash, hives or itching - Unusual bleeding or bruising
- Unusual tiredness or weakness - Yellowing of eyes or skin. Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.